



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متدد

إسم المريض رباعياً : عزيزه محمد بخيت رضوان
المحافظة : القاهرة الحي / المدينة : المقطم
العنوان : مساكن القاهرة - ب 11 - م2 المقطم
رقم البطاقة : 27812202600065 رقم التليفون : 01143852153
العمر (تاريخ الميلاد) : 20-12-1978 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 06-10-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإخصائي الاجتماعي
التوقيع: (.....) المقر بما فيه التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة في هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / أ.د. / أ.د. / د. /

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
جهة العلاج :
ملاحظات :
قسم :
نفقات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: عزيزه محمد يثيث رضوان
National ID: 27812202600065



تقرير طبي

المسيد

تحية طيبة وبعد,,

بشان تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بان نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :
PM 01:17 05-07-2026

تاريخ وساعة الدخول :
-- -- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد انه

المریضة تعاني من ورم غير حميد بالبدن و تحتاج الي Further investigations and imaging

الوقت :

-- --

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين



KM7160859



٢٧٨١٢٢٠٢٦٠٠٠٦٥

عزلة
محمد بن حبيب بن حنبل
مسكن القادر - ١١ - ب
المقام - القاهرة



٢٠٢٤/١١
٢٧٨١٢٢٠٢٦٠٠٠٦٥
حامل على الرقم ٢٧٨
التي
مسكن
مسكن القادر - ١١ - ب
المقام - القاهرة حتى ٢٠٢٤/١١/٢٢



١١٤٢٢٧٥٢١٥٢
١١٤١٥١٥٥٢١

جمهورية مصر العربية
بطاقة تعقب الشخصية

عزیزہ
محمد بخیت رضوان
مساكن القاهرة - ب ۱۱ - ۳
المقننم - القاهرة

۱۹۷۸/۱/۱۷
۲۷۸۱۲۲۰۲۶۰۰۰۶۵
KW7160859

۲۰۲۲/۱۱ ۲۷۸۱۲۲۰۲۶۰۰۰۶۵
حاصله على دبلوم تجاره

أنثى مسلمة متزوجة
عبدالقوى عبدالباسط عبدالقوى بخيت
البطاقة سارية حتى ۲۰۲۱/۱۱/۲۳

1

Name: عزيزة محمد بخيت رضوان		
MR. : 12073		
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالتها		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انها		
The patient is a 47-year-old individual who is post-menopausal. There is no reported family history of malignancy and the only noted comorbidity is M.FRR. Core needle biopsy of the right breast revealed invasive ductal carcinoma, grade 2 (RT. IDC G2). Immunohistochemistry demonstrated estrogen receptor positivity in 8/8 cores, progesterone receptor positivity in 3/8 cores, HER2 score of +1, and a Ki-67 proliferative index of 30 %. Imaging with sonomammography performed in July 2026 identified a right-upper-inner quadrant lesion measuring 4 cm from the nipple at the 10 o'clock position, with an associated 2 × 1.7 cm component and non-specific axillary lymph nodes. Staging workup with CT of the chest, abdomen, and pelvis and a bone scan were both negative for metastatic disease. The referring team recommended consideration of clipping followed by neoadjuvant chemotherapy (NACT) and then radiotherapy with wide local excision (WLE) and sentinel lymph node biopsy (SLNB). The multidisciplinary team agreed to proceed with clipping, administer four cycles of AC chemotherapy followed by four cycles of taxotere, and subsequently perform radiotherapy with WLE and SLNB.		
تحريراً في ,		
	التاريخ :	23-09-2026
	الوقت :	
	الطبيب المعالج :	
يعتمد ,		

Patient Name	عزيزه محمد بخت	Case No.	262277
Sex/Age	Female 47 Y	Patient No.	19970
Registered Date	19/5/2026 04:10:57 PM	Referred By Dr.	Interventional Radiology
Validation Date	11/6/2026 05:47:22 PM	Branch	Gustave Roussy
Reporting Date	01/7/2026 10:29:29 AM	Pathology No.	1592/26

Clinical Data: Right breast mass (BIRADS 4b).

Nature of Specimen: Tru-cut biopsy, collected on 19/05/2026, at 2:00 pm. Cold ischemia time is 20 seconds.

Gross Examination: Four grey, white tissue cores, ranging in size between 1.0 cm & 1.2 cm long, totally submitted.

Technique:

- Sections prepared from paraffin block for immune-staining using Benchmark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination:

- Histologic type: Invasive duct carcinoma.
- Histologic grade: 2 (differentiation 2, pleomorphism 2, mitosis 2)
- DCIS: Absent.
- Lobular cancerization: Present.
- Microcalcification: Absent.
- Angiolysis: Indefinite.
- Perineural invasion: Absent.
- Tumor infiltrating lymphocytes (TILs): Moderate.

Results:

- * IPT (All controls showed appropriate reactivity):
- ER: Strong nuclear reaction in 99% of tumor cells (score 8/8).
- PR: Moderate nuclear reaction in 1% of tumor cells (score 3/8).
- Her2: Faint incomplete membrane reaction in more than 10% of tumor cells (score 1+).
- Ki67%: 30%.

Conclusion:

Right breast mass, Tru-cut biopsy:

- Invasive duct carcinoma, Grade 2.

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 3/8).



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
 Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadima - Cairo, Egypt

Patient Name	عزيزه محمد بخيت	Case No.	262277
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Validation Date	11/6/2026 05:47:22 PM	Branch	Gustave Roussy
Reporting Date	01/7/2026 10:29:29 AM	Pathology No.	1592/26

- Her2: Low (score 1+).

- Ki67%: 30%.

****Scoring system: "ER, PR"**

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

**** Scoring system: "Her2"**

Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature

Dr. Walaa Ibrahim Ghoneim



غرفة رقم (4) - الدور الاول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
 Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Name: Aziza Mohamed

Date: 30 April 2026

MAMMOGRAPHY AND HIGH FREQUENCY ULTRASONOGRAPHY OF BOTH BREASTS

Indication for examination:

- Patient presenting with a painless right breast lump of 6 m duration.
- No previous exams available.

Findings:

- Overall breast composition: there are scattered areas of fibroglandular densities (ACR category b).
- Both breasts display normal skin thickness with normal appearance of the nipples and subareolar complex.
- Normal appearance of the subcutaneous and retromammary fat on both sides.
- The right breast shows an irregular spiculated dense mass in the upper inner quadrant with a suspicious cluster of microcalcifications. Complementary US confirmed the previous findings and revealed an irregular hypoechoic mass at 1 o'clock position, measuring 1.9 x 1.7 cm, 5 cm from the nipple, with spiculated margins, echogenic borders, posterior acoustic shadowing and showing internal vascularity.
- No left breast suspicious spiculated masses, architectural distortion or asymmetric densities. No suspicious clusters of microcalcifications are noted. Complementary US of the left breast confirmed the previous findings and revealed no suspicious cystic or solid lesions.
- No axillary lymphadenopathy.

Impression:

- Suspicious right breast mass as described above (BI-RADS 4 B) for Histopathological correlation

Best regards,
Dr. Ayman Amgad, MSc

جمهورية مصر العربية
بطاقة لتفلي الشخصية

عزيزه
محمد بخيت رضوان

مسكن القاهره - ب ١١ - ٢ م
المقطم - القاهره

١٩٧٨/١٢/٢٦
٢٧٨ ١٢ ٢٠ ٢٦ ٠٠ ٦٥

KW7160859

٢٠٢٤/١١ ٢٧٨ ١٢ ٢٦ ٠٠ ٦٥

حاصله على دبلوم تجاره

أنثى مسلمة متروجة

عبدالقوى عبدالباسط عبدالقوى بخيت

البطاقة سارية حتى ٢٠٢٤/١١/٢٦

9/13/26, 11:36 AM

code:09 - Name_A MDT FORM - Name_E MDT FORM

د. يوسف
روسي
رئيسة اللجنةمملكة العربية السعودية
لجنة صحة المرأة المصريةالهيئة العامة
للصحة
بمصرCib L
9-12

09/13/2026

MDT FORM

Name:	عبد الحفيظ رشيدان	MRN:	12073
Age:	48 Y	ID:	27812202600065
Menopausal Sta	POST	Phone:*	01143852153
NRS SCORE:		Marital Status:	
BMI:		Family History:	-VE FH
Gender		Reason Of MDT:	NEW CASE
CO Morbidity:	N.FRR	ECOG Performance Status	0 00 10 20 30 4
Surgical History:*	NAD		

Clinical Data:

R

L

BX: RT, IDC G2

ER: 8/8 PR: 3/8 HER2: +1 KI67: 30%

History:

T N M

Sonomammography:

7/2026 RT UIQ AT 1 OCLOCK 4 CM FROM NIPPLE 2*1.7 CM
NON SPECIFIC AXILLARY LN

Biomarker/Molecular

Mets Workup:

CT CAP >> FREE

BONE SCAN >> FREE

Recommendations:

FOR POSSIBILITY OF CLIPPING. NACT THEN RT WLE & SLNB

Clinical Staging:
CT2N0M0

MDT Outcome & Plan

FOR clipping

NACT AC * 4 THEN TAXOTERE *4

THEN RT WLE & SLNB

Pathological Staging:

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY

د. يوسف
روسي
رئيسة
الطبيب
السريري

قبط
9-12



مملكة العربية السعودية
لصحة المرأة المصرية

الهيئة العامة
للصحة
والتربية

09/13/2026

MDT FORM

Name:	اسم المريض	MRN:	12073
Age:	48 Y	ID:	27812202600065
Menopausal Sta	POST	Phone:*	01143852153
NRS SCORE:		Marital Status:	
BMI:		Family History:	-VE FH
Gender		Reason Of MDT:	NEW CASE
CO Morbidity:	N,FRR	ECOG Performance Status	0 00 10 20 30 4
Surgical History:*	NAD		

Clinical Data:

BX: RT, IDC G2
ER: 8/8 PR: 3/8 HER2: +1 KI67: 30%

History:

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7/2026 RT UIQ AT 1 O'CLOCK 4 CM FROM NIPPLE 2*1.7 CM
NON SPECIFIC AXILLARY LN

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Mets Workup:

CT CAP >> FREE
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Recommendations:

FOR POSSIBILITY OF CLIPPING NACT THEN RT WLE & SLNB

MDT Outcome & Plan

FOR clipping
NACT AC * 4 THEN TAXOTERE *4
THEN RT WLE & SLNB

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY



T N M

Q1 Q2 Q3 Q4 Q1 Q2 Q3 Q4
Right Side Left Side

Clinical Staging:

CT2N0M0

Pathological Staging:

الأستاذ الدكتور

عمر عبد الرحيم الطبري

أستاذ الجراحة العامة والأورام بكلية طب سوهاج
دكتوراه واستشاري جراحة الشرج والقولون وجراحة الأورام
عضو بجمعية الجراحين العرب والجمعية الأوروبية للجراحة

الأسم :
التاريخ : ٢٠٠٦ / ٧ / ٤
السن :

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الإستشارة خلال أسبوع بميعاد سابق

سوهاج : شارع المحطة - ميدان العروبة فوق مبنى العروبة أمام المطافى
٠١٠٣٢٩٩٨٩٠٠
٠٩٣/٢٣٥٧١١١

مواعيد العيادة : طوال أيام الأسبوع ما عدا الخميس والجمعة

MINISTRY OF HEALTH
SPECIALIZED MEDICAL CENTER
SOEHRAG CANCER CENTER
Radiology Department



وزارة الصحة والسكان
الهيئة العامة للطبية المتخصصة
مركز أورام سوهاج
قسم الأشعة التخصصية

Patient Name: azeza mohamed bakhet radwan	Study Date: 7/22/2026 8:55:04 AM
Patient ID: 83353092	Age:
Study Description:	

وزارة الصحة أمثلة المراكز الطبية المتخصصة مركز أورام سوهاج
CT Chest without IV Contrast

Status;

Pathology;

Comparison;

- ** Clear well aerated lungs parenchyma, no masses or nodules.
- ** No pleural effusion detected at both sides.
- ** Normal appearance of mediastinal structures.
- ** No mediastinal masses or LN enlargement in this non enhanced CT.
- ** Normal appearance of trachea and main bronchi.
- ** Free bony thoracic cage. No detectable soft tissue abnormality.

IMPRESSION

Within average non enhanced CT Chest Findings.

Dr/M.Mamdouh,MD



Patient Name	: السيدة / عزيزة محمد بخيت رضوان	Request No.	: 110367960
Patient No.	: 83353092	Request Date	: 30/07/2026
Path No.	: 3474/2026	Verification Date	: 03/08/2026
Sex	: Female	Financial Category	: نفقة دولة
Age	: 48 Y	Department	: خلرجي - سوهاج
Refereing Doc	: 1		

Pathology Report

CLINICAL DIAGNOSIS: Rt breast mass at UIQ opposite 1 o'clock

NATURE OF SPECIMEN: TCNB

GROSS EXAMINATION: The specimen consists of 4 tissue cores (3 cm long) collectively, grayish in color, totally submitted in one cassette.

MICROSCOPIC EXAMINATION: Sections revealed infiltration by atypical epithelial cells arranged in cords and nests, the cells show pleomorphism, hyperchromatic nuclei and rare mitosis, the stroma is desmoplastic

DIAGNOSIS: Labelled as Rt breast mass at UIQ opposite 1 o'clock, TCNB

- Features of invasive duct carcinoma, NST, grade II

Signature

Pathologist

Dr.Christeen Nady

Dr. Eman Azzam

N.B : Specimens received at the department of pathology are examined and sampled for diagnostic purposes. The specimens are normally held for four weeks and then disposed. A request for return of the specimens must be made at the time of surgery or within four weeks.

IMPRESSION

**Within Average Abdomen & Pelvis Ultrasound
Findings.**

Dr:Fatma Abdelbaset.



Patient Name: azeza mohammed bachet	Study Date: 7/30/2026 12:09:29 PM
Patient ID: 83353092	

وزارة الصحة
أمانة المراكز الطبية المتخصصة
مركز اورام سوهاج

Bilateral Breast Sonomammography

Status: New case with Rt breast lesion.

Rt BREAST

**** Evidence of rather defined iso to hypoechoic focal lesion with irregular outlines showing inner central degenerations seen at UIQ opposite 1 o'clock about 4 cm away from nipple, it measures +/- 2x1.7 cm.**

**** Otherwise, Homogenous rest of breast parenchyma with normal Fatty-glandular component.**

**** Normal appearance of lactiferous ducts.**

Rt Axillae: Few enlarged non specific Axillary LNs (oval shaped with preserved hilum) of average 1 cm.

Li BREAST

**** Homogenous breast parenchyma with normal Fatty-glandular component.**

**** No focal solid or cystic mass lesions Could be seen.**

**** Normal appearance of lactiferous ducts.**

Lt Axillae: Few enlarged non specific Axillary LNs (oval shaped with preserved hilum) of average 1 cm.

IMPRESSION

- Right breast UIQ focal lesion (BIRADs VI) as pathologically proved outside invasive duct carcinoma.

Abdomen and Pelvis Ultrasound

Liver: Average size. Normal texture .Smooth surface. No focal lesions. No intra hepatic biliary channel dilatation or venous congestion.

Portal Vein: Patent, Average Caliber.

Gall Bladder: Average capacity&wall thickness. Clear bile, noStones.

CBD: not dilated. No stones seen inside it.

Spleen: Average Sized. homogeneous echopattern. No focal lesions.

Pancreas: Average size and texture , no detected masses .

Rt Kidney: Normal site, size. Average Parenchymal thickness & echogenicity. No back pressure changes or calculi. No renal masses.

Lt Kidney: Normal site, size. Average Parenchymal thickness & echogenicity. No back pressure changes or calculi. No renal masses.

Urinary Bladder: Average capacity. Clear content. No stones or masses.

No Pelvic masses.

No ascites or abdominal masses.

No enlarged abdominal or Pelvic lymph nodes

جمهورية مصر العربية
بطاقة تحقيق المصلحة

عزیزہ
محمد بخیت رضوان

مسکن القاهره - ب ۱۱ - م ۳
المقننم - القاهره

٢٧٨ ١٢ ٢٠ ٢٦ ٠٠٠ ٦٥

KW7160859



Name: Aziza Mohamed

Date: 30 April 2026

MAMMOGRAPHY AND HIGH FREQUENCY ULTRASONOGRAPHY OF BOTH BREASTS

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- No axillary lymphadenopathy.

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Best regards,

Dr. Ayman Amgad, MSc

الاسم:

بشأن تقرير طبي للمريض:.

تاريخ وساعة الخروج.

وبيط الكشاف والفحوصات والمتابعة وجد انه:

1) $\frac{d}{dt} \left(\frac{1}{2} m v^2 \right) = \frac{1}{2} m \frac{d}{dt} (v^2)$

والله اعلم بالصواب

التاريخ: ك. ١٧ /

الطبيب المعالج

[Signature]

cc - Intelligence

Patient Name	عزيزه محمد بخت	Case No.	262277
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Reporting Date	01/7/2026 10:29:29 AM	Pathology No.	1592/26

- Her2: Low (score 1+).

- Ki67%: 30%.

**Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, $\geq 1\%$; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

** Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faintly/barely perceptible and within $< 10\%$ of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faintly/barely perceptible and within $> 10\%$ of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within $> 10\%$ of the invasive tumor cells or complete and circumferential membrane staining that is intense and within $< 10\%$ of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense $> 10\%$ of tumor cells.

Signature

Dr. Walaa Ibrahim Ghoneim



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - مركز تشخيص النقي - مصر الجديدة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Miter El Qadima - Cairo, Egypt

Patient's Name: Aziza Mohammed Bakheet Radwan
Date: 2026-08-22
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS
Newly diagnosed case of right breast cancer. Initial study for metastatic work up.

I-CHEST:

- Right breast ill-defined soft tissue lesion. Few axillary LNs, oval in shape, one shows no hilum measuring around 20x12mm for correlation with sonomammography.
- Right cardiophrenic LN measuring 10x8mm (non-specific).
- Clear both lung fields with no evidence of sizable pulmonary masses, nodules or cavitations.
- No mediastinal or hilar pathological enlarged lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal enhanced appearance of the thoracic aorta and the great vessels

II-ABDOMEN AND PELVIS

- Average size of the liver showing homogenous parenchymal attenuation. No focal lesion or dilated biliary radicles.
- Patent homogenously enhanced portal vein and its main branches.
- Normal CT appearance of both kidneys showing normal site, size and shape with smooth regular outline. No definite cysts, masses or back pressure changes.
- Normal CT appearance of the spleen, adrenal glands, pancreas, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- Average caliber of the bowel loops.
- Bone window reveals no suspicious lytic or sclerotic lesions.



OPINION:

- Newly diagnosed case of right breast cancer, current study reveals no suspicious metastatic deposits in the chest, abdomen or pelvis.

Shohenda. M. Taha, MD

Name : Aziza Mohammed Bakheet Radwan

Date : 20-08-2026 08:35

Branch : Dokki

Medical No. : 443691

Ref.Physician Prof : Goustav Roussey Egypt

NUCLEAR MEDICINE BONE SCAN REPORT

Clinical Indication: 47-year-old female with right breast cancer. Presents with right knee pain. Referred for baseline skeletal staging.

Procedure: Whole-body planar bone scintigraphy, anterior and posterior views, following intravenous administration of 99mTc-MDP.

Findings:

- Homogeneous physiological tracer distribution throughout the axial and appendicular skeleton.
- No focal areas of abnormal increased uptake suggestive of osteoblastic metastases.
- Mild symmetrical periarticular uptake in both knees and shoulders, consistent with degenerative/arthritis changes.
- Renal excretion is prompt and symmetric. No abnormal soft tissue uptake.

Impression:

- No scintigraphic evidence of skeletal metastases.

Much oblige
Dr. Alaa Nagi, MD
Nuclear medicine

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Gustave Roussy

Branch

Pathology No.

Pathology No.

Clinical Data: Right breast mass (Duke) 19/05/2026, at 2:00 pm. Cold ischemia time is 20 seconds.
Cold ischemia time is 20 seconds.

Specimen: Tru-cut biopsy, collected on 12/06/2012

Gross Examination: Four grey, white tissue

Technique: Embed in paraffin block for immunostaining using DAKO LSAB kit & Ki67 clone (30-9).

- Sections prepared from patient
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & NO

Microscopic examination.

- Microscopic examination:
- Histologic type: Invasive duct carcinoma.
- Histologic grade: 2 (differentiation 2, pleomorphism 2, mitosis 2)
- DCIS: Absent.
- Lobular cancerization: Present.
- Microcalcification: Absent.
- Angiolymphatic invasion: Indefinite.
- Perineural invasion: Absent.
- Tumor infiltrating lymphocytes (TILs): Moderate.

Results: All controls showed appropriate reactivity. (score 8/8).

- * IPI (All cancer patients):
- ER: Strong nuclear reaction in 99% of tumor cells (score 3/8).
- PR: Moderate nuclear reaction in more than 10% of tumor cells (score 1+).
- Her2: Faint incomplete membrane reaction in more than 10% of tumor cells (score 1+).
- Ki67: 30%.

Right breast mass, invasive ductal carcinoma Grade 2.

- ER: Positive (score 8/8).
- PR: Positive (score 3/8).

